FOUNDRY Rx

THE NEW RULES OF

Medical Weight Loss.

GLP-1s · Muscle Preservation · Peptide Support · Medical Sourcing

A complimentary Foundry RX guide to losing weight without losing strength, energy, or trust in the process.

GLP-1s changed weight loss. But medication alone is not the full protocol.

MOST PEOPLE START WITH ONE QUESTION:

How much weight can I lose?

THE BETTER QUESTIONS ARE:

— What am I actually losing?
— Who manages my dose?
— How do I protect muscle?
— What happens if side effects show up?
— Where does my medication come from?
— Will the body I end up with be stronger — or just smaller?

GLP-1 medications change appetite, cravings, blood sugar response, and the speed of weight loss. That is powerful. But the quality of the outcome is determined by what surrounds the medication: protein, resistance training, recovery, sourcing, titration, provider oversight, and — when appropriate — supportive co-therapy.

This guide exists for people who want to understand the full protocol before they begin. Not a shortcut. Not a checkout page. A smarter way to think about medical weight loss.

> *"The goal is not simply to lose weight. The goal is to lose the right weight."*
>
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What this guide covers.

"Muscle is metabolic infrastructure."

— FOUNDRY Rx

Before you spend a dollar, ask what you are buying.

In medical weight loss, the difference between a real protocol and a product page is not always obvious. A polished website can sell access. A real medical program has to do something harder: evaluate whether treatment is appropriate, prescribe responsibly, manage side effects, adjust dosing, and source medication through standards that can be explained without hesitation.

That distinction matters. You are not just choosing a medication. You are choosing the clinical system around it.

A reputable provider should be able to answer five questions clearly:

Who reviews my intake?

A licensed provider should review your health history, current medications, contraindications, and goals before any prescription decision is made. This is not optional. It is the baseline.

Where does the medication come from?

The pharmacy should be identifiable, licensed, and operating under clear, verifiable standards. If the source is vague, that is not a minor detail. It is a signal about the entire operation.

How is dosing managed?

GLP-1 treatment is not a static prescription. Dosing changes over time based on tolerance, side effects, appetite response, and progress. A platform that ships a fixed dose without follow-up is not managing a protocol.

What happens if I feel side effects?

Nausea, constipation, fatigue, reflux, and appetite suppression are not uncommon during GLP-1 therapy. A serious program explains how they are managed before they happen — not after you email a support address.

Am I being sold a protocol or just a vial?

This is the most important question. A vial is a product. A protocol is a clinical process with intake, oversight, adjustment, and follow-up. The two are not interchangeable.

> **FOUNDRY STANDARD**
> *A medical weight loss program should begin with provider review, not a checkout page. At Foundry RX, no prescription is issued without intake screening by a licensed clinician.*

GLP-1s do not create willpower. They change the signal.

GLP-1 stands for glucagon-like peptide-1, a hormone your gut naturally releases after you eat. It communicates simultaneously with your brain, your stomach, your pancreas, and your metabolic system — helping regulate hunger, fullness, blood sugar, and digestion.

Injectable GLP-1 medications such as semaglutide amplify this signal in ways the body cannot produce on its own. That is why many patients describe the experience in terms that sound almost strange at first: food becomes less loud. Cravings become less urgent. Portions become easier to control. The constant negotiation with appetite quiets down. This is not a personality change. It is biology.

MECHANISM 1 — GASTRIC EMPTYING

Food stays in the stomach longer. You feel full sooner and stay full longer. Blood sugar rises more gradually, smoothing the insulin response. This eliminates the blood sugar spike-and-crash cycle that drives afternoon hunger and impulsive eating in many people.

MECHANISM 2 — APPETITE SIGNALING

GLP-1 receptors in the hypothalamus help regulate hunger and reward-driven eating. When activated, the pull toward overeating diminishes — not because of willpower, but because the brain's hunger signal has been recalibrated. Many patients describe food becoming genuinely less interesting. Researchers call this reduction in "food noise."

MECHANISM 3 — INSULIN AND GLUCAGON REGULATION

GLP-1 medications help the body respond to rising blood sugar in a controlled, glucose-dependent way. They also suppress glucagon — the hormone that tells the liver to release stored glucose. The result is a more stable metabolic environment and improved insulin sensitivity over time.

MECHANISM 4 — THE CALORIC DEFICIT

When appetite decreases and fullness increases, most patients naturally eat less. This creates the sustained caloric deficit that drives weight loss. But this is precisely where the protocol matters most — because the medication cannot, by itself, guarantee that the weight lost comes primarily from fat.

THE CATCH — AND WHY IT MATTERS

GLP-1 agonists are extraordinarily effective at creating a caloric deficit. But the human body, losing weight rapidly, does not cleanly distinguish between fat and lean tissue. In the absence of deliberate countermeasures — adequate protein, resistance training, clinical monitoring, and appropriate co-therapy — a significant portion of total weight lost during GLP-1 therapy may come from lean mass: muscle, bone density, and connective tissue.

Clinical reviews have documented that between 25 and 40 percent of total weight lost during rapid pharmacologic weight loss can come from lean tissue rather than fat. That is not a footnote. It has direct consequences for metabolic rate, strength, injury risk, bone health, and long-term ability to maintain the results.

This is not a reason to avoid GLP-1 therapy. It is a reason to do it properly.

THE PROTOCOL AROUND THE MEDICATION

The medication handles appetite. The protocol handles everything else. That means protein targets. Resistance training. Dose pacing that prevents under-eating. Recovery support. And, for patients with specific clinical needs, thoughtful co-therapy. This is why the Foundry model pairs every GLP-1 protocol with the oversight and support layers that determine what the weight loss actually looks like.

"The medication handles appetite. The protocol handles everything else."

RESEARCH NOTE

Semaglutide (STEP trials) produced average body weight reductions of 14-17% over 68 weeks. Tirzepatide (SURMOUNT trials) reached 20-22% in some cohorts — comparable to bariatric surgery. The next clinical frontier is not simply total weight lost, but the composition of that loss: fat versus lean mass, strength retention, and long-term metabolic function.

Not all weight loss is equal.

Two people can lose thirty pounds and end up with completely different bodies, completely different metabolic rates, and completely different long-term outcomes.

One loses primarily fat, protects muscle, keeps training, and is metabolically stronger than before they started. The other loses fat and muscle together, feels weaker, recovers poorly, burns fewer calories at rest, and becomes significantly more likely to regain the weight.

Same number on the scale. Completely different outcome. This distinction is what the weight loss industry has spent decades avoiding.

WHY MUSCLE IS NOT A COSMETIC CONCERN

Muscle is metabolic infrastructure. It is not optional tissue. Here is what it actually does:

Glucose disposal: Skeletal muscle is responsible for approximately 80 percent of insulin-stimulated glucose uptake in the body. When you lose muscle, you lose the body's primary mechanism for clearing blood sugar after meals — meaning insulin resistance can increase even as the scale drops.

Resting metabolic rate: Lean mass is metabolically expensive tissue. The body burns significantly more calories maintaining muscle than fat. Losing ten pounds of muscle can reduce resting metabolic rate by 80 to 120 calories per day — a gap that compounds over months into the structural driver of weight regain.

Bone and connective tissue: Rapid weight loss accelerates bone resorption, particularly without mechanical loading from resistance training. As body weight decreases, ligaments and tendons experience new mechanical demands. Without structural muscle support, injury risk climbs.

Strength and quality of life: Loss of lean mass affects how you look, how you move, your energy levels, your training capacity, and your ability to maintain the result you worked for.

Protecting muscle during GLP-1 treatment requires deliberate action across five areas:

PROTEIN
1.2 to 1.6 grams per kilogram of body weight per day. This is the raw material for muscle repair. Most people in a GLP-1-induced caloric deficit are eating less — and often far less protein than their muscles require.

RESISTANCE TRAINING
The body preserves what it uses. Without a training stimulus, there is no signal to retain lean tissue during a caloric deficit. Even two sessions per week produces measurable lean mass preservation.

TITRATION
Dosing should be managed so that appetite suppression does not become under-eating. Aggressive deficits accelerate lean mass loss. A provider who monitors intake and adjusts dosing protects composition, not just weight.

RECOVERY
Sleep quality directly affects cortisol levels. Elevated cortisol accelerates lean mass catabolism — meaning poor sleep during GLP-1 therapy actively works against the protocol. Hydration, electrolytes, and inflammation management matter more when the body is changing quickly.

CLINICAL SUPPORT
A provider should monitor response, side effects, and tolerance over time. Lean mass loss is not always visible on a standard scale. Body composition tracking and provider review make the invisible visible.

"The goal is not smaller. The goal is stronger, leaner, and more metabolically resilient."

A complete protocol moves four levers.

GLP-1 therapy moves one: appetite. Outcomes are determined by the other three.

01 MEDICATION

APPETITE CONTROL

GLP-1 medication, when clinically appropriate, prescribed after provider review. Reduces food noise. Creates the caloric deficit. Manages hunger and cravings.

02 TRAINING + NUTRITION

MUSCLE PRESERVATION

Protein (1.2-1.6g/kg daily), resistance training, dose pacing. Protects metabolic rate. Preserves strength. Reduces rebound risk.

03 SLEEP + RECOVERY

RECOVERY CAPACITY

Sleep, hydration, electrolytes, inflammation management. Supports tissue repair. Controls cortisol. Enables adaptation between sessions.

04 CLINICAL SUPERVISION

MEDICAL OVERSIGHT

Provider review, titration, side-effect management, sourcing standards. Manages what the medication cannot. Determines the composition of weight lost.

Medication creates the opportunity. The protocol determines the outcome.

What your body may be telling you during weight loss.

Symptoms are not failures. They are feedback. A good protocol addresses them proactively.

WHAT YOU FEEL	POSSIBLE PROTOCOL FOCUS	SUPPORT CATEGORY
Low energy, fatigue	Calories, hydration, mitochondrial support	Cellular + energy
Weak workouts, slow recovery	Protein intake, resistance training, recovery co-therapy	Growth hormone axis
Joint discomfort	Connective tissue support, training adjustment	Repair + structural
Poor sleep	Dose timing, stress management, restoration	Sleep support
Skin quality changes	Collagen support, hydration, rate of loss review	Regenerative
Lean mass concern	Protein, resistance training, titration review	Co-therapy eval
GI discomfort, nausea	Dose titration, meal timing, side-effect management	Provider-managed

All co-therapy at Foundry RX is provider-selected, prescription-based, and sourced from a 503B-registered pharmacy.

CLINICAL NOTE

Nausea, constipation, appetite suppression, and fatigue are not uncommon during GLP-1 treatment, particularly during early dose titration. A protocol should include a clear plan for managing side effects before they occur — not an email address to contact after the fact.

Peptides are not magic. They are signals.

Peptides are short chains of amino acids. In the body, they often act as messengers — helping cells communicate with specific systems involved in repair, recovery, metabolism, sleep, inflammation, and tissue remodeling. That specificity is what makes them therapeutically interesting.

They are not a replacement for nutrition, training, sleep, or medical oversight. They are a support layer — one that becomes relevant when patients on GLP-1 therapy experience symptoms that the medication alone does not address.

Not every patient needs co-therapy. The decision should always be provider-led, based on health history, goals, contraindications, and how the patient responds over time. What follows is a framework for how a clinician might think about it.

IF THE ISSUE IS LOW ENERGY OR FATIGUE

Caloric restriction reduces substrate availability. Mitochondrial function can decline during sustained deficits. When fatigue does not resolve with adequate sleep and hydration, a provider may evaluate cellular or mitochondrial support pathways — including NAD+ and related protocols.

IF THE ISSUE IS WEAK WORKOUTS OR SLOW RECOVERY

Muscle protein synthesis requires both training stimulus and nutritional support. In a caloric deficit, recovery slows and strength decreases — which accelerates lean mass loss. A provider may evaluate growth hormone axis support (such as CJC-1295 / Ipamorelin or Sermorelin) as part of a lean mass preservation protocol.

IF THE ISSUE IS JOINT DISCOMFORT

As weight redistributes rapidly, joint loading changes. Ligaments and tendons adapt under new mechanical stress. BPC-157 and TB-500-class protocols are used clinically to support connective tissue repair and recovery.

IF THE ISSUE IS SLEEP DISRUPTION

GLP-1 therapy can affect sleep architecture during early titration. Caloric restriction independently affects slow-wave sleep — the phase most critical for tissue repair and growth hormone secretion. DSIP and related restoration protocols may be considered.

IF THE ISSUE IS SKIN QUALITY CHANGES

Rapid fat loss — particularly in the face — can reduce collagen support and skin firmness. GHK-Cu and related regenerative protocols are used to support skin quality and collagen pathways during active weight loss.

Weight loss changes how you look. Metabolic health changes how you age.

Most people enter medical weight loss focused on the scale. That makes sense — it is visible and feels objective. But as the body changes, patients often become aware of deeper questions: Why do I feel tired? Why does my skin look different? Why do I feel lighter but not stronger?

This is where weight loss becomes a longevity conversation. Rapid fat loss reveals the quality of the tissue underneath. Muscle tone, collagen support, hydration, sleep, and inflammation all determine how the result looks and feels over time.

MUSCLE
Strength, metabolic rate, glucose disposal, long-term independence.

SKIN + COLLAGEN
Elasticity, texture, how fat loss shows in the face over time.

"The best version of weight loss does not make you look depleted. It makes you look restored."

Weight loss changes how you look. Metabolic health changes how you age.

Most people enter medical weight loss focused on the scale. That makes sense — the scale is visible, gives feedback, and feels objective.

But as the body changes, patients often become aware of deeper questions: Why do I feel tired? Why does my skin look different? Why are my workouts harder? Why do I feel lighter but not stronger? How do I maintain this?

This is where weight loss becomes a longevity conversation.

Rapid fat loss reveals the quality of the tissue underneath. Muscle tone, collagen support, hydration, sleep, and inflammation all influence how the result looks and feels long-term.

MUSCLE
Strength, metabolic rate, glucose disposal, posture, and long-term independence.

SKIN + COLLAGEN
Elasticity, texture, and the way fat loss shows in the face and body over time.

CELLULAR ENERGY
Mitochondrial health — the driver of training capacity, recovery, and daily vitality.

SLEEP
The repair window where hormones, tissue repair, and recovery are coordinated nightly.

INFLAMMATION
The difference between feeling lighter on the scale and actually feeling better.

"The best version of weight loss does not make you look depleted. It makes you look restored."

The source is part of the treatment.

This is the section the peptide industry would prefer you skim.

The peptide and weight loss market has a sourcing problem. The problem is not just whether a product has the right name on the label. The problem is whether the product has verified identity, accurate potency, confirmed sterility, and traceable handling standards — from raw material to your dose.

For injected therapies, this matters at a different level of urgency. When a compound is injected, it bypasses the body's primary filtering systems — the digestive tract, which metabolizes and limits absorption of many contaminants. Purity, sterility, and accurate concentration are not premium features. They are the baseline requirement for any compound that enters the bloodstream directly.

A compound with the right name on the label but wrong concentration, unverified purity, or compromised sterility is not the same therapy. It is a different thing — and the risk profile is entirely different.

THE 503B STANDARD

In the United States, compounding pharmacies operate under two regulatory frameworks. 503A pharmacies compound for individual patient prescriptions under state pharmacy board oversight. 503B outsourcing facilities are federally registered with the FDA, required to operate under current Good Manufacturing Practices (cGMP), subject to mandatory sterility testing, and held to documentation standards comparable to pharmaceutical manufacturers. 503B is the higher standard — and the one Foundry RX partners with exclusively.

WHAT TO VERIFY BEFORE YOU BEGIN

Clear pharmacy identity: The platform should disclose exactly where medication is prepared and under what regulatory framework.

503B registration: For injected therapies, this matters. Ask directly.

Batch testing: Testing should evaluate identity, potency, sterility, endotoxin levels, and heavy metal content — not just one or two of those criteria.

Certificate of Analysis: A serious provider can explain whether batch documentation exists and how quality is verified on every batch — not just on request.

Provider supervision: Sourcing standards do not replace medical oversight. They support it.

> **FOUNDRY STANDARD**
> *Foundry RX partners exclusively with 503B-registered compounding facilities. All raw materials are USA-sourced. Every batch is independently tested for identity, potency, sterility, endotoxin levels, and heavy metals. Certificate of Analysis available on request.*

A peptide store is not a medical protocol.

Ten signs the platform you are evaluating is selling a product, not managing a protocol.

01 No provider review or health intake before purchase

02 No prescription decision — checkout available to anyone

03 No contraindication or medication history screening

04 Pharmacy identity is unclear, unlisted, or offshore

05 Products labeled "research use only" or "not for human use"

06 No licensed prescriber named or reachable for questions

07 No batch testing documentation or CoA available

08 Side-effect support and dose titration not included

09 Before/after claims without clinical context or caveats

10 **You are being sold a vial — not a protocol.**

"If you cannot verify the source, you cannot fully understand the risk."

Diet programs ask for discipline. Medical protocols change the condition

Most diet programs are built on a simple premise: the problem is behavior. Eat less. Move more. Track everything. Try harder.

That advice can work for some people, for some period of time. But it often ignores the biology underneath appetite — insulin resistance, reward-driven eating, sleep disruption, stress hormones, and metabolic adaptation. It treats a signaling problem as a character problem.

Medical weight loss starts from a different premise: the body is not a calculator. It is a signaling system. GLP-1 medications work because they change signals — hunger, fullness, digestion, blood sugar, cravings, and reward. The results reflect that difference. Clinical trial data shows weight loss outcomes that were previously achievable only through surgery.

But changing the signal is only one part of the system.

WHY PROVIDER SUPERVISION MATTERS

A complete medical protocol considers whether the patient is eligible. How aggressively to dose. How to manage side effects. How to preserve lean tissue. How to support recovery. How to adjust when the body changes. How to prevent the result from becoming temporary.

That is not something a checkout page can do. Physiology changes over time. A protocol should evolve with the person — not run on autopilot until the subscription lapses.

The distinction between a supervised protocol and a self-directed one is not about distrust of patients. It is about respecting that metabolism, body composition, tolerance, and response are dynamic — and that clinical judgment, applied over time, is what separates durable outcomes from temporary ones.

WHAT THE EVIDENCE SHOWS

GLP-1 therapy without behavioral and nutritional support produces meaningful but often temporary results. Patients who discontinue therapy without preserving lean mass and building sustainable habits frequently regain weight rapidly — sometimes more than they originally lost. The medication changes the entry conditions. The protocol determines whether the outcome lasts.

"Medical weight loss is not willpower with a prescription. It is physiology with oversight."

A real protocol starts before the prescription.

The right process should feel clear and simple — but not casual. Here is what it actually looks like at Foundry RX.

STEP 1 — MEDICAL INTAKE
You provide health history, current medications, goals, contraindications, and relevant lifestyle context. This is not a sign-up form. It is a clinical intake.

STEP 2 — PROVIDER REVIEW
A licensed provider reviews whether GLP-1 treatment or peptide support is clinically appropriate for you specifically. Not everyone is a candidate. That is the point.

STEP 3 — PRESCRIPTION, IF APPROPRIATE
Medication is prescribed only when clinically indicated. The prescription decision is made by a provider, not by an algorithm or an automatic approval system.

STEP 4 — PHARMACY PREPARATION
Medication is prepared through our 503B pharmacy partner under strict sourcing and quality standards. USA-sourced raw materials. Batch-tested. Certificate of Analysis available.

STEP 5 — DELIVERY AND INSTRUCTIONS
Medication ships discreetly with instructions for dosing, storage, administration, and what to expect during early titration.

STEP 6 — TITRATION AND FOLLOW-UP
Your provider adjusts the protocol based on appetite response, side effects, weight loss, energy, and tolerance. Dosing is managed — not set and forgotten.

STEP 7 — CO-THERAPY, IF APPROPRIATE
Peptide or recovery support may be added when the clinical picture calls for it — fatigue, lean mass concern, joint discomfort, sleep disruption, or skin quality.

STEP 8 — LONG-TERM PLAN
The goal is not just starting. It is building a result your body can maintain — with the metabolic foundation, lean mass, and habits to support it.

"A vial

is not

a protocol."

— THE FOUNDRY STANDARD

If GLP-1 treatment or peptide support is appropriate for you, it should start with a medical review —

not a checkout page.

BEGIN YOUR PROVIDER REVIEW

foundryrx.co

Physician-reviewed

503B Compounded

USA Sourced

"The goal is not smaller. The goal is stronger, leaner, and more metabolically resilient."

Medical Disclaimer: This guide is for educational purposes only and does not replace medical advice.
Always consult a qualified healthcare provider before beginning any therapy. Individual results vary.

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